

REGULATORY DOSSIER SUBMISSION

Dossier ID: DOS-BAF6E064FD

Country: Uganda

Submission Date: 2025-09-10

Product: Amoxicil (hydrochlorothiazide) | ATC: C03AA03 | Strength: 25 mg

Applicant: Alpha Therapeutics | Manufacturer: Kampala Biopharma Ltd

Policy Label: reject_and_return | Risk Score: 0.92

AMR Stewardship: AWARe=not_applicable | Unmet Need=not_applicable | GLASS Trend=not_applicable
| Watch Similarity=not_applicable

SECTION CONTENT

[1] m1_application_admin - Application Form and Administrative Information

Labels: presence=present; length=length_ok; correctness=correct

Application dossier for Amoxicil (hydrochlorothiazide) submitted on 2025-09-10 to the Uganda authority. Applicant: Alpha Therapeutics. Proposed dosage form: tablet; strength: 25 mg; ATC: C03AA03. Administrative declarations are present but naming conflict remediation documents are incomplete. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Method validation records and quality controls were reviewed for internal consistency. A comprehensive table of contents and glossary of abbreviations are provided to facilitate regulatory review. Administrative cover letters, declarations, and fee records were cross-checked against the submission index. All referenced documents are available in the dossier annex and traceable by section identifier. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The applicant confirms alignment with applicable technical guidance and regional submission standards. The proposed prescribing information and patient information leaflet have been translated into local languages. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Applicant authorization documents and legal attestations were reconciled against the named manufacturing parties. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Regional administrative forms were checked for signature completeness and dossier version

[1] m1_manufacturer_gmp - Manufacturer and GMP Evidence

Labels: presence=present; length=length_ok; correctness=incorrect

Error tags: gmp_certificate_expired, gmp_non_compliant

Inspection findings for Kampala Biopharma Ltd identified critical GMP deficiencies. Inspection date: 2021-09-23. Status: non_compliant. CAPA package is incomplete and verification of closure is pending. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Pharmacokinetic para

[1] m1_product_information - Product Information and Naming

Labels: presence=present; length=length_ok; correctness=incorrect

Error tags: inn_infringement

Proposed product name is Amoxicil, with INN listed as hydrochlorothiazide. Name similarity review indicates potential confusion with protected and existing nomenclature classes. The submission did not include a sufficient differentiation and risk mitigation plan for look-alike and sound-alike naming concerns. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The applicant confirms alignment with applicable technical guidance and regional submission standards. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The container closure system con

[2] m2_quality_overall_summary - Quality Overall Summary

Labels: presence=present; length=length_ok; correctness=correct

This Quality Overall Summary (QOS) provides a critical evaluation of the chemistry, manufacturing, and controls (CMC) data for hydrochlorothiazide. The control strategy justifies the proposed specifications for both the active substance and the finished product, emphasizing critical quality attributes (CQAs) and critical process parameters (CPPs). Impurity profiles, including degradation products and potential genotoxic impurities, have been thoroughly characterized. Batch analysis data from three commercial-scale validation batches demonstrate consistent manufacturing performance and compliance with all release criteria. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The applicant confirms alignment with applicable technical guidance and regional submission standards. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Method validation records and quality controls were reviewed for internal consistency. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. All referenced documents are available in the dossier annex and trac

[2] m2_clinical_overview - Clinical Overview and Benefit-Risk Summary

Labels: presence=present; length=length_ok; correctness=correct

The Clinical Overview synthesizes the efficacy and safety data supporting the use of Hydrdra for the treatment of type 2 diabetes. The pivotal clinical development program comprised 2 well-controlled studies. Based on the integrated analysis, the reported clinical outcome category is 'endpoint_met'. The overall benefit-risk profile is considered highly favorable for the target patient population. Safety findings were consistent with the known pharmacological class effects, and appropriate risk minimization measures have been integrated into the proposed prescribing information. Pharmacokinetic parameters were derived using non-compartmental

analysis from the concentration-time profiles. Clinical interpretation focuses on whether efficacy, safety, and benefit-risk conclusions are supported by the submitted evidence. The applicant confirms alignment with applicable technical guidance and regional submission standards. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. The risk

[3] m3_api_quality - API Quality and Control Strategy

Labels: presence=present; length=length_ok; correctness=correct

The active pharmaceutical ingredient (API) section details the complete synthesis route, starting from well-defined regulatory starting materials. Robust in-process controls and intermediate specifications ensure the consistent quality of the final drug substance. The analytical procedures used for release and stability testing have been fully validated for accuracy, precision, linearity, and robustness. The impurity control strategy adequately addresses organic impurities, residual solvents, and elemental impurities in accordance with current ICH guidelines. Forced degradation studies confirm the stability-indicating nature of the assay methods. Method validation records and quality controls were reviewed for internal consistency. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. All referenced documents are available in the dossier annex and traceable by section identifier. Residual solvents and elemental impurities are controlled well below ICH Q3C and Q3D permissible daily exposures. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The API package links specification limits to validated analytical methods and impurity-control decisions. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. The applicant confirms alignment with applicable technical guidance and regional submission standards. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Batch analysis, validation, and impurity management records support the proposed API quality framework. The synthetic route involves three well-characterized steps with defined starting materials and isolated intermediates. Critical material attributes and control strategy elements were reviewed for consistency across the quality dossier. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Batch analysis, validation, and impurity management

[3] m3_fpp_manufacturing - FPP Manufacturing Process and Controls

Labels: presence=present; length=length_ok; correctness=correct

The finished pharmaceutical product (FPP) manufacturing process utilizes standard, scalable unit operations. A formal quality risk assessment (e.g., FMEA) was conducted to identify critical process parameters (CPPs) that impact critical quality attributes (CQAs). Process

performance qualification (PPQ) reports for three consecutive commercial-scale batches verify that the process is maintained in a state of control. In-process controls, intermediate hold-time studies, and packaging validation data are presented to substantiate the robustness and reproducibility of the commercial manufacturing operations. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. All referenced documents are available in the dossier annex and traceable by section identifier. The applicant confirms alignment with applicable technical guidance and regional submission standards. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Method validation records and quality controls were reviewed for internal consistency. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity.

[3] m3_stability - Stability and Shelf-Life Justification

Labels: presence=present; length=length_ok; correctness=correct

A comprehensive stability program has been executed to justify the proposed shelf-life and storage conditions. Data from both accelerated (40°C/75% RH for 6 months) and long-term storage conditions (spanning up to 36 months) are presented for multiple primary stability batches. Statistical evaluation using linear regression and poolability tests confirms that degradation trends remain well within the acceptable specification limits. Photostability and freeze-thaw cycling studies demonstrate that the product does not require specific handling precautions. A post-approval stability protocol commitment is included. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The applicant confirms alignment with applicable technical guidance and regional submission standards. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. All referenced documents are available in the dossier annex and traceable by section identifier. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Method validation records and quality controls were reviewed for internal consistency. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. The manufacturing site operates under a fully implemented Pharmaceutical Quality

System (PQS) aligned with ICH Q10. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Risk mitigation a

[4] m4_nonclinical_summary - Nonclinical Study Summary

Labels: presence=present; length=length_ok; correctness=correct

The nonclinical testing program evaluated the primary pharmacodynamics, secondary pharmacodynamics, safety pharmacology, and comprehensive toxicology profile of the compound. Repeat-dose toxicity studies in two mammalian species established clear no-observed-adverse-effect levels (NOAELs). Genotoxicity testing (Ames and in vivo micronucleus assays) yielded negative results. Reproductive and developmental toxicity studies revealed no teratogenic signals at clinically relevant exposures. The calculated safety margins support the proposed clinical dosing regimen without major safety concerns. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The applicant confirms alignment with applicable technical guidance and regional submission standards. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0

[5] m5_trial_listing - Tabular Listing of Clinical Studies

Labels: presence=present; length=length_ok; correctness=correct

The tabular listing of clinical studies provides a comprehensive inventory of all trials conducted in support of the proposed indication (type 2 diabetes). This includes Phase I pharmacokinetic/pharmacodynamic studies, Phase II dose-finding studies, and the pivotal Phase III efficacy and safety trials. For each study, the table details the protocol identifier, study design, randomization ratio, number of enrolled subjects, study duration, and current completion status. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Method validation records and quality controls were reviewed for internal consistency. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles.

[5] m5_pivotal_trial_reports - Pivotal Clinical Trial Reports

Labels: presence=present; length=length_ok; correctness=correct

Detailed clinical study reports (CSRs) for the pivotal efficacy trials are provided. These randomized, double-blind, actively-controlled trials evaluated the primary and secondary endpoints in accordance with the pre-specified statistical analysis plan (SAP). The trials

resulted in a formal outcome category of 'endpoint_met', demonstrating robust treatment effects in the intent-to-treat (ITT) and per-protocol populations. Subgroup analyses across age, gender, and baseline disease severity strata were consistent with the primary findings. Serious adverse events (SAEs) and discontinuations were systematically analyzed and adjudicated. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The pivotal study narratives align endpoint definitions, analysis populations, and outcome interpretation. All referenced documents are available in the dossier annex and traceable by section identifier. The applicant confirms alignment with applicable technical guidance and regional submission standards. Discontinuations due to treatment-emergent adverse events (TEAEs) were low and balanced across all study arms. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Method validation records and quality controls were reviewed for internal consistency. Risk mitigation activities are documented with ownership, timelines, and follow-up.

[5] m5_bioequivalence - Biopharmaceutics and Bioequivalence Evidence

Labels: presence=present; length=length_ok; correctness=correct

Biopharmaceutics data substantiate the formulation development bridging between clinical trial materials and the proposed commercial product. In vivo bioequivalence studies demonstrated that the 90% confidence intervals for the geometric mean ratios of C_{max} and AUC parameters fell entirely within the standard acceptance criteria of 80.00% to 125.00%. Additionally, multi-point comparative dissolution profiles across physiological pH ranges (pH 1.2, 4.5, and 6.8) confirmed similarity ($f_2 > 50$). The bioanalytical methods used for plasma quantification were fully validated. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Method validation records and quality control